NUR 234 · EXAM 3 · HIGH-YIELD REVIEW

High-Yield Review

The most-tested concepts from your study guide, condensed: priority assessments, nursing interventions, safety precautions, and the exam traps to avoid. Section numbers point back to the full study guide.

⚑ First actions to know cold

MAG TOXICITY
Stop the infusion

OXYTOCIN + TACHYSYSTOLE / BAD STRIP
Turn off oxytocin

CORD PROLAPSE
Lift the presenting part off the cord

SHOULDER DYSTOCIA
McRoberts + call for help

ORTHOPNEA (CARDIAC)
Sit her up

SUSPECTED ABRUPTION
Left side, O2, 2nd large-bore IV

PREVIA BLEEDING
Assess bleeding + FHT (no vaginal exam)

BP ≥ 160/110
Antihypertensive within ~30 min

NEWBORN OF DIABETIC MOM
Feed within 30 min, heel stick

UTERINE RUPTURE
To the OR now

HYPOTONIC LABOR + FULL BLADDER
Empty the bladder

AMNIOTOMY
FHT before and after

Module 8 · Pregnancy complications (Ch 20)

PRIORITY ASSESSMENT	NURSING INTERVENTIONS	SAFETY PRECAUTIONS	COMMON EXAM TRAP
Cardiac disease in pregnancy · guide 8.1			
VS often; lung sounds (crackles); dyspnea/orthopnea; edema; daily weight. Highest strain 28–32 wks and right after placenta delivery.	Rest (2 periods/day + full night); low sodium; meds exactly as prescribed; HOB up + left tilt; epidural; assisted vaginal delivery; continuous FHR; SCDs; stool softeners.	Never supine in labor. Watch for cardiac collapse/HF right after the placenta delivers (fluid shift). Left-sided failure is worse for the fetus.	Left = lungs (crackles, orthopnea); JVD/ascites are RIGHT-sided. Constipation and nausea at 8 wks are normal; 4 lb in a week is not. HF drugs aren't tested.
Venous thromboembolism (DVT) · guide 8.3			
Unilateral calf swelling, warmth, redness, tenderness; compare calf diameters. Ultrasound, PTT/INR.	Ambulate (best), hydrate, SCDs, prophylactic anticoagulant for high-risk post-cesarean.	Bleeding on anticoagulants; delivery may be timed with the drug stopped ~24 h before.	Anticoagulants do NOT dissolve the clot. D-dimer is unreliable in pregnancy.
UTI / pyelonephritis · guide 8.4			
Urine dip every visit (nitrites, WBC, glucose, protein). Pyelo: flank pain on tapping over kidney, fever, uterine irritability.	Pyelo = inpatient IV antibiotics, fetal monitoring, stay until afebrile and not contracting. Teach: wipe front to back, void after sex, fluids, cotton, no bubble baths.	Asymptomatic UTI can become pyelo, sepsis, and preterm labor.	Back labor is rhythmic, back-to-front; pyelo pain is over the kidney and reproduced by tapping. "Holding urine" and bubble baths are wrong answers.
Diabetes / GDM · guide 8.5			
Screen everyone 24–28 wks (earlier if high risk). 1-h > 140 → 3-h test; 2 abnormal values = GDM. Newborn: heel-stick glucose; jittery, lethargic, poor feeding.	Carb control (complex carbs), ~30 min exercise daily, glucose diary, insulin adjustments in 2nd/3rd trimester, growth US, NSTs, kick counts.	Newborn hypoglycemia: feed within 30 min of birth. Macrosomia → shoulder dystocia risk. GDM → higher lifetime type 2 risk.	Insulin does NOT cross the placenta (glucose does). One abnormal value ≠ GDM. Type 1/2 clients don't get a GTT. Values are given; don't memorize the chart.

Module 9 · Sudden pregnancy complications (Ch 21)

PRIORITY ASSESSMENT	NURSING INTERVENTIONS	SAFETY PRECAUTIONS	COMMON EXAM TRAP
Vaginal bleeding / spontaneous abortion · guide 9.0–9.1			
Gestational age; amount (count pads), color, clots, tissue; pain; Rh type; shock signs (↓BP, ↑HR, pale, clammy).	"Come in to be examined and save any tissue." Sort SAB by tissue lost. RhoGAM if Rh-negative.	Missed abortion/retained stillbirth → infection, septic shock, DIC.	Bed rest, a tampon, or "spotting is harmless" are wrong. Threatened = closed cervix, FHT present (most hopeful).
Ectopic pregnancy · guide 9.2			
Sudden one-sided lower abdominal pain + shock signs (case: BP 95/58, HR 132).	Ruptured → surgery. Unruptured → methotrexate: stop prenatal vitamins/folic acid, no alcohol, serial hCG, wait months to conceive.	Ruptured ectopic = hemorrhage emergency. RhoGAM if Rh-negative.	IUD, PID, tubal ligation, endometriosis are risk factors. Methotrexate is only for UNruptured.
Molar pregnancy (GTD) · guide 9.3			
Very high hCG, uterus larger than dates, dark brown "sludgy" bleeding, grape-like tissue, early preeclampsia signs.	D&C with suction; serial hCG follow-up.	Delay pregnancy 6 months–1 year; retained tissue can regrow (choriocarcinoma).	A rising hCG during follow-up can't be told apart from a new pregnancy, which is why she must wait.
Cerclage (cervical insufficiency) · guide 9.4			
Painless dilation history; signs of labor or ROM.	Nothing in the vagina, no tub soaks, take it easy.	Remove at 37 wks, or early if labor or ROM (or the cervix tears). Abdominal cerclage → cesarean.	Cerclage is for painless dilation, not labor.
Previa vs abruption · guide 9.5			
Previa: painless bright red, soft uterus. Abruption: painful dark red, board-like uterus, FHR drifting down → bradycardia.	Previa: assess bleeding + FHT, CBC, T&S. Abruption: left side, O2, 2nd large-bore IV, CBC/T&S/coags, emergent delivery.	NO vaginal exams with previa. Abruption can be concealed; DIC risk.	A slowly falling FHR baseline with bloody show can be an early abruption. Complete previa = cesarean.
DIC · guide 9.6			
Petechiae, purpura, oozing IV sites, gum/nose bleeding. Labs: platelets ↓, fibrinogen ↓, PT/PTT ↑.	Volume expanders, blood products, keep warm, ICU care.	Confirm type & screen, CBC, coags are actually drawn before procedures.	Fibrinogen is LOW, not high. D-dimer isn't reliable in pregnancy.
Preterm labor · guide 9.7			
Regular contractions, pelvic pressure, cervical change, nitrites (infection). fFN, UA, cultures.	Hydrate, treat infection. Terbutaline subcut (buys time). Betamethasone IM before 34 wks (2 doses, 24 h apart).	Hold terbutaline if maternal HR > 120 (cardiac boxed warning).	Terbutaline doesn't mature lungs; betamethasone does. A positive fFN does NOT mean she will deliver.
Preeclampsia / HELLP / magnesium · guide 9.8			
BP, headache not relieved by Tylenol, vision changes, epigastric pain, brisk DTRs/clonus, urine output, RR, LOC. HELLP: AST/ALT ↑, platelets ↓.	BP ≥ 160/110 → antihypertensive within ~30 min. Mag: low stimulation, IV fluids ≤ 125 mL/h, VS q30 min, strict I&O, continuous FHR.	Mag toxicity: STOP infusion → mag level → calcium gluconate → provider. PPH risk after birth. Platelets < 100,000 = no epidural.	Warm and flushed is EXPECTED on mag, not toxicity. The first action is stopping the drip, not giving calcium.

PRIORITY ASSESSMENT	NURSING INTERVENTIONS	SAFETY PRECAUTIONS	COMMON EXAM TRAP
Rh / amniotic fluid / twins · guide 9.9–9.11			
Maternal Rh type and Coombs. Oligo < 300 mL (renal); poly > 2000 mL (GI).	RhoGAM at 28 wks, within 72 h of birth if baby Rh+, and any time blood may mix.	Fluid problems raise cord prolapse, PPH, and prolonged labor risk.	RhoGAM protects FUTURE pregnancies. Identical twins share a placenta (higher risk).

Module 10 · Labor & birth complications, surgical interventions (Ch 23–24)

PRIORITY ASSESSMENT	NURSING INTERVENTIONS	SAFETY PRECAUTIONS	COMMON EXAM TRAP
GBS · guide 10.1			
GBS status from ~36-wk swab (sooner in preterm labor).	Penicillin in labor every 4 h; ≥ 2 doses before birth = covered.	Not covered → increased newborn surveillance for infection.	GBS is NOT an STI. Swab even if a cesarean is planned.
Power problems / dysfunctional labor · guide 10.2			
Contraction frequency, duration, resting tone; bladder; dilation (stage 1) vs descent (stage 2).	Hypotonic: empty bladder, oxytocin, amniotomy. Hypertonic: stop oxytocin, fluids, reposition, terbutaline if it persists.	Full bladder blocks descent, weakens contractions, raises hemorrhage risk. Goal: q2–3 min with ≥ 60 s rest.	Stage 1 problem = dilation; stage 2 problem = descent. Arrest = halted despite everything.
Oxytocin / cervical ripening · guide 10.4–10.5			
Before oxytocin: head engaged, vertex, ripe cervix (Bishop ≥ 8), category I/II strip, no CPD. Contractions and FHR q30 min.	Own pump, LR main line, port closest to the client. Titrate up if q5 min; leave if q2–3 min. Prostaglandin first if Bishop is low.	Stop for > 5 contractions/10 min, a contraction > 2 min, or a bad strip. Remove Cervidil (tape) for tachysystole/distress. Watch water intoxication.	Never piggyback oxytocin. "It is not a race." Apply the Bishop chart, don't memorize it.
Uterine rupture / inversion / AFE · guide 10.6			
Rupture: sudden excruciating pain (prior cesarean scar). Inversion: excessive bleeding. AFE: sudden dyspnea, cyanosis, arrest, DIC.	Rupture → OR, O2, fluids, blood. Inversion → provider replaces uterus. AFE → emergency response, DIC care.	Avoid excess fundal pressure and pulling on the cord (inversion).	Inversion is never normal. Sudden acute pain in labor is never normal.
Umbilical cord prolapse · guide 10.7			
Sudden FHR drop after ROM; cord felt or seen. Risk: high station at ROM, preterm, polyhydramnios, transverse.	Lift presenting part off the cord and hold; call for help; O2; prepare emergency cesarean; saline-soaked towel on exposed cord.	Don't get her up if ruptured with a high station.	Knee-chest is NOT the first action; it's a temporary band-aid. Don't let go until the OR.
Shoulder dystocia / macrosomia · guide 10.8			
Risk: GDM, macrosomia > 4000 g, post-dates. After birth: arm movement (Erb's), clavicle crepitus.	Call for help + NICU; McRoberts; suprapubic pressure when directed; record head and body times.	~5 min before serious hypoxia; be ready to resuscitate.	NEVER fundal pressure.
Malpresentation / ECV / CPD · guide 10.9–10.10			
Position (OP, breech, transverse, face/brow); pelvimetry for inlet/outlet contraction.	ECV: empty bladder, US in room, FHT, terbutaline, towel under right hip. Trial of labor if CPD borderline.	ECV needs ≥ 37 wks, adequate fluid, single fetus, reactive NST. Transverse lie → cord prolapse risk.	ECV is scheduled, not done in labor. Breech, transverse, face/brow usually → cesarean; OP can deliver vaginally.

PRIORITY ASSESSMENT	NURSING INTERVENTIONS	SAFETY PRECAUTIONS	COMMON EXAM TRAP
Forceps / vacuum · guide 10.11			
Full dilation and effacement, ROM, head engaged, bladder empty.	Empty bladder before the device; assess newborn for bruising, cephalohematoma.	Max 3 vacuum pop-offs. Only a skilled provider uses them.	9 cm is NOT enough (a 9 cm forceps case hemorrhaged).
Cord & placenta anomalies · guide 10.12			
Vasa previa: red blood with ROM + sudden bradycardia. Two-vessel cord at delivery.	Vasa previa: surveillance, scheduled cesarean. Two-vessel cord: work up for cardiac/renal anomalies.	Accreta spectrum → major hemorrhage; percreta may need hysterectomy.	Vasa previa blood is FETAL. Accreta = endometrium, increta = myometrium, percreta = through.
Amniotomy / internal monitoring / scalp stim · guide 10.13–10.15			
FHT before and after AROM; fluid color, odor, amount, time. FSE/IUPC need ROM + ≥ 3 cm.	Be ready for meconium. IUPC allows amnioinfusion.	AROM → cord prolapse, AFE. Internal devices → infection, limited mobility.	Most important AROM step = FHT. Scalp stim: an acceleration is reassuring.
Cesarean birth · guide 10.16			
Indications: scheduled (CPD, malpresentation, HSV, prior classical), emergency (distress, abruption, prolapse, arrest).	NPO after midnight, teaching, IV, prep; SCDs; early ambulation.	Hemorrhage, infection, organ injury; newborn respiratory issues and cold stress.	Classical incision = no vaginal birth later (rupture risk).

! Common exam traps: don't fall for these

FROM LECTURE + STUDY GUIDE

The wrong answers that keep showing up, drawn from the professor's NCLEX-style examples and the study guide.

1. Choosing a normal pregnancy change (nausea, constipation, frequency, foot edema) as the "abnormal" finding.
2. Treating the monitor instead of the patient.
3. Mixing up left (lungs) and right (body) heart failure.
4. Thinking insulin crosses the placenta.
5. Diagnosing GDM from one abnormal GTT value.
6. Telling a possible miscarriage to stay home on bed rest instead of coming in.
7. Doing a vaginal exam on a client with placenta previa.
8. Calling warmth/flushing on magnesium a sign of toxicity.
9. Giving calcium gluconate before stopping the magnesium.
10. Giving terbutaline with maternal HR over 120.
11. Expecting terbutaline (not betamethasone) to mature fetal lungs.
12. Reading a positive fFN as "she will deliver."
13. Expecting high fibrinogen or trusting D-dimer in DIC.
14. Thinking anticoagulants dissolve the clot.
15. Increasing oxytocin when contractions are already every 2–3 minutes.
16. Piggybacking oxytocin or connecting it far from the client.
17. Positioning first (knee-chest) instead of lifting the head off a prolapsed cord.
18. Fundal pressure during a shoulder dystocia.
19. Forceps or vacuum before full dilation.
20. Treating vasa previa bleeding as maternal blood.
21. Calling GBS a sexually transmitted infection.
22. Offering a trial of labor after a classical incision.

★ PROFESSOR EMPHASIZED

Her test-taking rule: **know your normals**, and the wrong answers are easier to eliminate. Charts are given; you apply them. Study what is on the slides.